

FORM 4 — APPLICATION IN EXISTING PROCEEDINGS

QUEENSLAND INDUSTRIAL RELATIONS COMMISSION — *Workers' Compensation and Rehabilitation Act 2003 (Qld)* — Matter No: WC/2024/227

BETWEEN: CORY LEA SHEPHERD (Appellant) **and** WORKERS' COMPENSATION REGULATOR (Respondent)

APPLICATION IN EXISTING PROCEEDINGS (*Industrial Relations (Tribunals) Rules 2011, r 64G*)

OVERVIEW

This is an application under r 64G for the Commission to decide MSH's objection to a sealed Notice of Non-Party Disclosure. The Appellant has narrowed the Notice, withdrawn what the record does not need, and seeks production by a single self-executing step — a server-side export of records MSH already holds, bounded by identified custodian, date-range and phrase-family, requiring no review or reading of any email. Where a record is said not to exist, the Appellant asks only that the non-existence be stated on oath after a defined search. The objection therefore reduces to one question: whether that bounded export and produce-or-swear verification should be ordered, or whether a non-party's un-sworn assertion that its emergency-code paging logs and fatigue records "do not exist" should instead be accepted. The grounds below address relevance, possession, proportionality, and each ground of objection in turn; the relief sought is set out at Part B as the draft orders sought.

PART A — APPLICANT

Applicant: Cory Lea Shepherd, Appellant (Self-Represented). This application is supported by the Affidavit of Cory Lea Shepherd affirmed [date] ("the Affidavit"), and is made in respect of the Notice of Non-Party Disclosure (Form 29) sealed by the Industrial Registry on 22 April 2026 ("the Form 29"), served on Metro South Hospital and Health Service ("MSH") as the affected non-party.

PART B — DISPOSITION SOUGHT

The Appellant respectfully applies, pursuant to r 64G of the *Industrial Relations (Tribunals) Rules 2011*, for the objection of MSH to the Form 29 to be decided as follows:

(a) Pursuant to r 64G(2), the Notice be upheld as varied in the terms of Schedule A (the pressed Items being 1, 2, 3(a), 3(b), 9, 10 and 19 — Item 10 confined, per Schedule A, to (a) an aggregate metadata-

only count and (b)–(d) the phrase-family and bounded-correspondence exports), the objection being otherwise overruled, and MSH be ordered to produce the documents within that varied scope by native electronic export of the messages and records returned by the server-side filters described in the Form 29 (identified custodians, identified date-ranges and identified phrase-families) — no inbox review, reading or relevance-assessment of any email being sought or required.

(b) Verification affidavit (produce-or-swear). MSH is ordered, by its Chief Executive or a delegated Director, to file and serve a single affidavit within seven (7) business days of the order which, in respect of each pressed Item (Items 1, 2, 3(a), 3(b), 9, 10 and 19) and each Item on which MSH asserts non-existence (Items 3(c), 4, 5, 7, 14 and 20): (i) confirms on oath that the server-side export or search was undertaken, identifying the custodian, system, date-range and method applied; (ii) produces the records returned by that search; (iii) where the search returns no record, states on oath that the document does not exist, deposing the basis on which it is said not to exist; and (iv) is given by the Chief Executive or delegated Director as the accountable deponent, who may depose to matters within the knowledge of a systems or eHealth officer — in particular, for the SPOK emergency-code records (Items 1–2), the date and nature of the system upgrade, whether the pre-upgrade records were migrated, archived or destroyed, and the identity, author, holder and source system of the "spreadsheet of recorded MET calls" referred to in MSH's letter — on information and belief from a named officer, identifying that officer's position and exhibiting or separately swearing that officer's account.

(c) To the extent any extension is required, the time for the production at (a) is fixed at a date certain and not contingent upon the supply of any pleading to a non-party.

(d) Privacy safeguard. If any question of privacy or confidentiality is raised in respect of the export, the documents be produced to the Commission for its inspection in the first instance, with access to the parties as the Commission directs and subject to any order under s 580 of the *Industrial Relations Act 2016*.

(e) Confidentiality of sensitive material (s 580 IR Act 2016) — a single order in three limbs. *Limb 1 (medical/clinical):* non-publication of, and sealing on the Commission file with access restricted to the Commission and the parties (and any representative), of — the Appellant's medical certificates, the Work Capacity Certificates and the psychiatric report, and any health information; the monthly-statistics exhibit (CS-8) to the extent it records emergency-code data referable to clinical events (not extending to the aggregate call and code figures stated in the Affidavit); and any patient-identifying information in any material produced. (The Appellant notes Commissioner Dwyer's indication of 13 March 2026 that the unredacted medical records would be sealed if they came onto the record, and the Respondent's consent to that course; the order is sought on the footing that no perfected sealing order has yet been entered.) *Limb 2 (statutory secrecy — PID):* non-publication of any information identifying the fact or content of the Public Interest Disclosure (reference 24-ESU-1130) and the Ethical Standards Unit

determination, giving effect to the confidentiality required by s 65 of the *Public Interest Disclosure Act 2010* (Qld). *Limb 3 (confidential by agreement)*: a notation that the witness-conferencing material before the Commission is received subject to its existing confidentiality and is not to be published.

(f) Costs reserved.

(g) Such further or other orders as the Commission considers appropriate.

SCHEDULE A — DISPOSITION OF THE TWENTY ITEMS

The Appellant does not press the Form 29 in full. He has narrowed and disposed of the twenty Items as follows, each pressed Item anchored to the stressor pleaded in the Amended 9A:

Pressed (production sought):

Item	Document	Relevance anchor
1	SPOK emergency paging export, 17–18 March 2024 — Code Blue/MET activations and doctor-on-call paging events, with event timestamp, code type and response-acknowledgement timestamp (clinician/patient identifiers may be redacted)	Stressor 3 + 1(a): the objective record of which clinicians were paged and whether the pages were received, across the admitted 7-hour-break shift pairing
2	Aggregate SPOK activation counts (monthly, by code type), Logan Switchboard, stressor period	Stressor 1(a)/3: objective volume of emergency codes coordinated
3(a)	Complaints corpus — the misrouted/wrong-team/paging-error complaints made to, or managed by, the Line Manager and Director: (i) the messages themselves, and (ii) an aggregate monthly count showing the volume of those complaints and any increase across the pleaded period (sender, date and subject only — no bodies). These complaints were received at first instance at the Switchboard and dealt with there before any escalation (Affidavit ¶¶19–21)	Stressor 1(a): dereliction of clinical governance; delay to urgent pathology results; the scale and trend of the conduct across the course

Item	Document	Relevance anchor
3(b)	Management discussion of those complaints and what action (if any) was taken on them — minutes, agendas, action registers, and any record of remedial, procedural or safety action	Stressor 1(a): whether the conduct was addressed (cf. MSH's "no consequential changes", Item 3(c))
9	The Line Manager's own security access-card records (by her card identity) for the collocated office area in which the Switchboard sits, and building access, confined to the pleaded-conduct windows (the 9 May 2024 directive; the 15–21 May 2024 retraction correspondence; the August 2023 grievance period). No switchboard-office swipe is sought; the records sought are the Line Manager's own card swipes into the area and building, which MSH's access-control system holds	Stressor 1(a): to corroborate the manager's on-site availability on the identified occasions a directive issued or a complaint was left unactioned; not an audit of her movements generally
10	Targeted manager email export by server-side filter: (a) an aggregate monthly count of Line-Manager → Switchboard-list messages (sender, date, recipient-list and subject only — no message bodies), to show volume without producing the corpus; (b) emails from the Line Manager's email account to the Logan Hospital Switchboard generic (shared) mailbox in which "effective immediately" appears in the subject line or the body (Affidavit ¶26) — a single sender-to-mailbox, server-side filter on the phrase-family identified in the Form 29, which a simple export will satisfy; (c) the operational-directive phrase-family (the "wrong ward/team/doctor", "pager error", "out of date", "update the list" vocabulary at Affidavit ¶30); and (d) Manager → Director correspondence concerning the Appellant's retracted email and the comparator email of 9 May 2024 (bounded to those events)	Stressor 1(a), (f): unilateral directives without consultation; disparate treatment
19	Union-delegate correspondence: from the Appellant's 18 May 2024 request to Together (Ms Heather Hayes) to the injury date, 18 June 2024: the union's forward of that request to MSH ("your Health team") and MSH's internal handling/response	Stressor 1(g): MSH received the delegate request and did not act on it through to injury onset; QH-POL-248 positive duty (Form

Item	Document	Relevance anchor
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24, Paras 17–18)

Produce-or-swear (Items 1–2 SPOK; and Items 3(c), 4, 5, 7, 14, 20). The Appellant presses Items 1 and 2 on the footing that the SPOK records exist and should be produced. As to each Item on which MSH's letter asserts non-existence (3(c), 4, 5, 7, 14, 20), the Appellant neither accepts nor rejects that informal assertion; he asks only that it be placed on oath under Part B(b) after a defined search — so that each Item is either produced or its non-existence is sworn. A sworn statement that a hospital does not retain the paging logs of its emergency codes (Items 1–2), or holds no fatigue risk assessment for the 17–18 March 2024 pairing (Item 4), is of obvious significance to the s 32(5)(a) inquiry; the Appellant reserves any argument on the effect of a sworn negative until the verification affidavit is filed.

Already produced / resolved (no order needed) — Items 6, 11, 12, 13, 15, 16. These were disclosed with MSH's letter of 5 June 2026, or in the Respondent's disclosure of 11 June 2026 (Affidavit, ¶51–52); no order is sought.

Withdrawn — Items 8, 17, 18.

Correction to Item 19 (date and scope). Item 19 of the Form 29 anchored the union-delegate matter to a text message of 5 March 2024 and a date range commencing 1 March 2024. That was an error. The disclosure the Appellant presses is confined to the period from his request to the Together union (Ms Heather Hayes) on 18 May 2024 to the date of injury onset, 18 June 2024 — capturing the union's forward of that request to MSH and MSH's handling (or non-handling) of it up to the injury. The earlier notification of intention to become a delegate (11 August 2023, Form 24, Para 18) and the failure to facilitate the appointment for a period exceeding 9 months (Form 24, Para 19) are pleaded and admitted and do not require disclosure. This narrows Item 19; it does not expand it. MSH's objection that Item 19 would require searching the emails of its ~20,006 staff mischaracterises the Item: it was always confined to messages received by identified custodians (the Line Manager, the Director, the Logan Hospital health-team mailbox and HR) — not a search of all staff. The narrowed scope above is confined to those custodians — which is the very limitation MSH itself proposed in its letter of 5 June 2026.

PART C — GROUNDS

1. **The matter in issue.** The Form 29 is directly relevant to the matter in issue in the appeal (the s 32(5)(a) "reasonable management action" defence) as articulated in the Amended Statement of Facts and Contentions filed 7 April 2026 ("the Amended 9A"). The Continuation Schedule to the Form 29 states that matter in issue and ties each Item, by a Relevance Anchor Table, to specified stressors

pleaded in the Amended 9A and to the admitted facts in the Respondent's Form 24.

2. The health dimension. The documents sought are not collateral to a private dispute; they go to whether the patient-safety and clinical-governance functions of the Switchboard were compromised. As deposed (Affidavit, paras 6–25), the Switchboard is the hospital's communications hub — the point at which emergency codes are activated and urgent pathology results are relayed doctor-to-doctor. The misrouted-call corpus (Item 3(a)), the manager's presence record (Item 9), and the manager's directives (Item 10) bear directly on whether those time-critical clinical and emergency communications were delayed or misdirected. The Commission's evaluation of the s 32(5)(a) defence is, in this matter, inseparable from the patient-safety reality the records will reveal — and that reality is presently visible only from within MSH's own systems.

3. Possession and control. As deposed (Affidavit, para 46), each pressed Item is recorded in documents within MSH's exclusive possession or control — its email system, its Active Directory / Entra / Citrix identity-management environment, its Security Access swipe-card system, its meeting and governance records, and its roster and HR systems. The Appellant, as the Switchboard operator, cannot produce them.

4. No other reasonably simple, inexpensive way (r 64B). The probative content of the documents is their completeness — the scale, frequency and pattern of the conduct pleaded. The Appellant can produce individual examples (and has annexed them to the Affidavit), but cannot export the complete corpus. There is no other reasonably simple, inexpensive way for the Appellant to prove what the documents show, save by bulk server-side export from MSH's systems, which is achievable at minimal cost and requires no inbox review.

5. The period is the course of conduct — not a narrow window, and not unlimited. Because the injury developed cumulatively over the course of management conduct, the documents go to the volume and frequency of that conduct. The Form 29 noticed the email-based Items (3(a), 3(b) and 10) for the period 1 December 2023 to 30 June 2024, and the Appellant presses no period earlier or broader than the Notice; the SPOK paging Item (Item 1) is confined to 17–18 March 2024. Confining disclosure to those two days alone would sample the conduct, not show it; the period the Notice fixes is the appropriate measure of the pleaded course (and see the global evaluation of a composite course in *Delaney v Q-COMP* [2005] QIC 11; 178 QGIG 197). The volume of directives from the Line Manager to the Switchboard across the noticed period is sought not by producing the corpus but by an aggregate monthly count (sender, date, recipient-list and subject only — no message bodies: Schedule A, Item 10), which proves the scale without any review of content (Stressor 1(a)).

6. The fatigue-governance records (Items 4, 5 and 7) are records the applicable fatigue framework requires to be held. As a person conducting a business or undertaking, MSH owes a primary duty under the *Work Health and Safety Act 2011* (Qld) to manage risks to health and safety

— which include fatigue — by assessing and recording them. MSH's own letter states that fatigue risk management at the Switchboard was "implemented... after 30 June 2024 in connection with an organisational change related to the reporting lines for Switchboard" — which concedes that a fatigue risk-management framework applies to the Switchboard work area. That framework reflects the Queensland Health Fatigue Risk Management System (FRMS Implementation Guideline QH-GDL-401-3.3) and the fatigue-leave policy QH-POL-135, which expressly extends to "Administrative stream employees" engaged under the *Hospital and Health Service General Employees (Queensland Health) Award* — that is, to Switchboard staff. MSH's objection that fatigue records do not exist because "Fatigue Risk Management System training only applies to health practitioners and clinical assistants" answers the wrong question: Item 5 concerns training, but Items 4 and 7 concern the fatigue risk-assessment and register records, which the framework requires regardless of any training threshold and regardless of whether the staff are clinical. Whether those records exist for the Switchboard as at 17–18 March 2024 is precisely what the verification on oath (Part B(b)) should establish.

7. The objection cannot rest on non-existence. MSH itself produced documents responsive to Items 6, 11, 12, 13, 15 and 16 with its 5 June 2026 letter — including my myHR leave records and AVAC history for Process Reference 15480560 (Item 11) — which establishes that the classes of record sought are real and retrievable from MSH's systems; and on 11 June 2026 the Respondent disclosed further responsive material drawn from within MSH's systems (the 15–16 May 2024 correspondence falling within the Item 20 window). The Appellant has further annexed individual examples (including the manager's email of 9 May 2024 and the union correspondence of 18–20 May 2024) which demonstrate, on their face, that the classes of document sought are real.

8. The application is not oppressive — it seeks an export, not a review. The Appellant does not seek the review, reading, or relevance-assessment of any email by MSH. He seeks only the native electronic export of the messages and records returned by a server-side filter applied to the identified custodians, identified date-ranges and identified phrase-families set out in the Form 29. The filter is self-executing and requires no inbox review of any kind. Each Item is bounded at the system level; the relief is the antithesis of a general search; and it is offered subject to nil-result discharge (the verification affidavit at Part B(b)(ii)). Should MSH assert legal professional privilege over any specific record returned by the filter, it may do so item-by-item, which does not require, and is not a basis for resisting, the export itself. Further, if any question of privacy or confidentiality is raised, the Appellant consents to the export being produced to the Commission for its inspection in the first instance (Part B(d)), with access to the parties as the Commission directs. No question of privacy or privilege can therefore justify refusing the export — at most it governs the manner of access, which is the Commission's to manage.

9. The verification affidavit is the proportionate course. Framed as produce-or-swear, the order leaves MSH a single, bounded step: it either produces the documents, or verifies their non-existence

on oath after a defined search. Either course resolves the objection without an item-by-item production dispute. The power to make the order arises under r 64G(2), which empowers the Commission to make any order appropriate to its decision of the objection, together with the Commission's general power to give directions for the conduct of the proceeding; the order does no more than ensure the objection — production, or verified non-existence — is resolved on a proper evidentiary footing. The Appellant respectfully submits this is the orderly and proportionate disposition.

10. The model-litigant expectation on a government respondent. In *State of Queensland (Queensland Police Service) v Workers' Compensation Regulator* [2025] QIRC 203 (O'Neill IC), the Commission, in a disclosure dispute, applied the model-litigant principles to a State respondent. While that duty binds the Regulator (and the State) as a party, the Appellant respectfully submits that the same expectation of candour and cooperation should inform how a government entity such as MSH approaches a bounded, self-executing server-side export: it should facilitate, not resist, production, and a bare assertion that its emergency-code paging logs or fatigue records "do not exist" should be tested on oath rather than accepted.

11. Evidentiary context. The Respondent obtained the Appellant's primary medical records under a Notice of Non-Party Disclosure (Form 29) that was never served on the Appellant; the Respondent admits the non-service and relies on s 64D as removing any obligation to serve him (Form 24, Para 25). Whatever the merit of that defence, the contrast is instructive: the very non-party-disclosure process MSH now resists is the process the Appellant has followed to the letter — sealed Notice, service, and this application — whereas his own records were obtained without the Notice ever reaching him.

PART C2 — MSH'S OBJECTIONS ANSWERED (letter of 5 June 2026, K-LM26/729, signed by the Chief Executive)

MSH's objections reduce to three refrains, each of which the Notice already answers — and the letter itself supplies several of the answers the disclosure was designed to obtain.

1. *"We have not received the Form 9A, so we cannot assess relevance."* The Notice does not depend on the supply of the parties' pleadings to a non-party. The Continuation Schedule states the matter in issue and ties each Item, by a Relevance Anchor Table, to a specified Stressor and an admitted Form 24 fact on its face (r 64C). Relevance is demonstrable from the Notice itself; the absence of the Form 9A in MSH's hands is not a ground under r 64E(4).

2. *"Compliance would require eHealth to review and search individual staff email accounts" (Items 3(a), 10).* It would not. The Appellant seeks a server-side export (a self-executing filter bounded by identified custodian, identified date-range and identified phrase-family) and expressly disclaims any

inbox review (Notice, Administrative Concessions; Part B(a)). MSH's "expense and inconvenience" objection rests on a characterisation of the request as a manual account-by-account search, which is incorrect. As to Item 3(a) in particular, MSH's own letter records that complaints about Switchboard operational errors "are made directly to the Line Manager of Switch Board and managed solely via email" — i.e. a single identified mailbox, not a hospital-wide trawl.

3. *"The matter in issue relates only to the consecutive shifts of 17–18 March 2024."* It does not. The 7-hour break (Stressor 3) is one of three pleaded stressors; Stressor 1(a) pleads a course of conduct. The Form 29 noticed the email-based Items (3(a), 3(b) and 10) from 1 December 2023 to 30 June 2024; confining disclosure to two days would sample that course, not show it (*Delaney v Q-COMP* [2005] QIC 11).

4. *"Item 19 would require searching the emails of ~20,006 staff."* It would not. Item 19 is confined to identified custodians (the Line Manager, the Director, the Logan Hospital health-team mailbox and HR) and is narrowed to 18 May – 18 June 2024 (Schedule A): the very limitation MSH itself proposed in the letter.

5. *"Item 9 (building access) lacks relevance and particularity, and the Switchboard office has no swipe access."* The Appellant does not seek a switchboard-office swipe. He seeks the Line Manager's own security access-card records (by her card identity) for the collocated office area and the building — records MSH's access-control system holds — confined to the pleaded-conduct windows (Schedule A). Particularity is satisfied (one identified person; identified dates); relevance is direct (whether the manager was on site when a directive issued or a complaint was left unactioned, going to Stressor 1(a)); and any effect on the Line Manager is managed by the Commission-inspection-first safeguard and the s 580 order sought (Part B(d)–(e)), which governs the manner of access, not refusal.

6. **The letter supplies the answers, under the Chief Executive's hand.** In responding, MSH has stated: (a) *No remedial change* — "there have been no 'consequential' changes to operating procedures over the period requested" (Item 3(c)): despite the misroute complaints, nothing was changed. (b) *No fatigue framework at the time, and a restructure* — fatigue risk management at the Switchboard "occurred after 30 June 2024 in connection with an organisational change related to the reporting lines for Switchboard" (Item 5): no FRMS framework was in place at the date of injury. (c) *Emergency-code logs not retained* — the SPOK records "do not exist... records predating [the system] upgrade were not retained," while "a spreadsheet of recorded MET calls is available for the period 17–18 March 2024" (Items 1–2). The Appellant cannot tell from MSH's description whether that "spreadsheet" is the monthly call-statistics record he himself authors and maintains (Exhibit CS-8; Affidavit, ¶9) or a separate MSH or SPOK extract. If it is a separate MSH record, it is a contemporaneous record for the exact pleaded shift pairing and should be produced under Item 1, not treated as resolved; the verification at Part B(b) should identify what the spreadsheet is, who created

and holds it, and the system from which it derives. These are not reasons to refuse production; they are matters directly relevant to the s 32(5)(a) inquiry.

7. The verification affidavit should formalise these statements. Where MSH states a record does not exist (Items 1–2, 3(c), 4, 5, 7, 14, 20), the order at Part B(b) requires that to be stated on oath by the Chief Executive or a delegated Director, with the search identified — so the statements made informally in the letter are placed on the evidentiary footing the appeal requires. In particular, the verification should address, on oath: (i) Items 1–2 (SPOK): the date and nature of the system upgrade, and whether the pre-upgrade emergency-code paging records were migrated, archived or destroyed, and the basis on which they were "not retained" — an upgrade not ordinarily entailing the disposal of a hospital's emergency-code logs; (ii) Item 3(c): the "no consequential changes" statement, reconciled with the changes the letter itself records (the SPOK upgrade, the reporting-line restructure and the issue of a Queensland Health device) each occurring after the injury; (iii) Items 4, 5 and 7: what fatigue-management framework governed the Switchboard as at 17–18 March 2024 (see ground 6); and (iv) Item 20: MSH states PID 24-ESU-1130 "commenced in November 2024", whereas the underlying complaint was lodged on 13 May 2024 (Affidavit, ¶34; the determination of that complaint as a Public Interest Disclosure on 24 December 2024 being admitted at Form 24, Para 20). The verification must identify whether records of the operational and HR response in the 13–31 May 2024 window exist — particularly because the Respondent itself disclosed, on 11 June 2026, correspondence dated 15–16 May 2024 falling within that very window (Affidavit, ¶52). MSH cannot swear that the Item 20 material "does not exist" consistently with that disclosure: the verification must therefore either produce the records, or contradict its own 5 June 2026 letter or the Respondent's own disclosure — the reference number being a later-assigned identifier, not the date the underlying material came into existence.

PART D — MATERIAL RELIED UPON

- The Affidavit of Cory Lea Shepherd affirmed [date], and the exhibits annexed thereto ("CS-1" to "CS-8").
- Referred to and relied upon, not annexed (already before the Commission / disclosed in the proceeding): MSH's letter of 5 June 2026 (ref K-LM26/729), and the Respondent's disclosure of 11 June 2026.
- The Form 29 sealed 22 April 2026 and its Continuation Schedule.
- The Amended Statement of Facts and Contentions; the Respondent's Form 24 (Notice to Admit Facts) and the facts admitted thereunder.

FILED BY: Cory Lea Shepherd, Appellant (Self-Represented) — **DATE:** [date]

SCHEDULE B — RELEVANCE MAP

Each pressed Item is tied below to the document sought, the matter in issue pleaded in the Amended 9A, the admitted fact in the Respondent's Form 24, and the s 32(5)(a) element it tends to prove or disprove — the demonstration, on the face of the application, that it is the documents (not merely the allegations) that are directly relevant (*State of Queensland (Department of Families) v Workers' Compensation Regulator* [2025] QIRC 179 [46]–[48], applying the directly-relevant test to non-party disclosure in a psychological-injury appeal; and, by analogy only, the cognate principle under the UCPR in *Chenoweth v ING* [2004] QSC 143).

Item	Document sought	Matter in issue (Amended 9A)	Anchor (Form 24 admitted facts)	What the document tends to prove on s 32(5)(a)
1	SPOK emergency-paging export, 17–18 Mar 2024 (code/MET activations; response-acknowledgement timestamps)	Stressor 3: managing life-safety codes under the 7-hour-break exhaustion	Para 1 (7-hr break); Para 34 (Review Decision contents admitted — incl. the "unreasonable management action" finding — relevance reserved); Para 8 (Switchboard patient-safety function)	Whether the rostering, and the safety-critical load coordinated under exhaustion, was reasonable management action taken in a reasonable way
2	Aggregate SPOK activation counts (by code type)	Stressors 1(a)/3: the volume of emergency coordination the role bore	Para 8 (patient-safety function admitted)	The weight and criticality of the role against which the management conduct must be assessed
3(a)	Misroute / "wrong team" / "pager error" complaints corpus + an aggregate monthly count showing volume and any increase across the period (server-side filter, named custodians); received at first instance at the Switchboard	Stressor 1(a): dereliction of clinical governance; delay to urgent pathology results	Para 5 (Reese's rostering-error acknowledgment); Item 3(c) "no consequential changes"	Whether the unilateral directives bottlenecked emergency workflows, the scale and trend of the conduct, and whether management's response was reasonable

Item	Document sought	Matter in issue (Amended 9A)	Anchor (Form 24 admitted facts)	What the document tends to prove on s 32(5)(a)
	(Affidavit ¶¶19–21)			
3(b)	Management discussion of those complaints and what action (if any) was taken (minutes, action registers, remedial-action records)	Stressor 1(a): whether the conduct was ever addressed	Item 3(c) "no consequential changes" (the admitted absence of remedial action)	Whether management acted reasonably on known clinical-governance failures
9	Security-access swipe-card / building-access records, confined to the pleaded-conduct windows	Stressor 1(a): manager's on-site availability on identified occasions	The manager-availability complaints (CS-5; Affidavit ¶20)	Whether the manager was present/available when a directive issued or a complaint was left unactioned, not an audit of her movements
10	Manager email by server-side filter: (b) "effective immediately" (subject or body; Line-Manager account → Switchboard shared mailbox) + (c) operational-directive phrase-families; (d) Manager → Director re the retracted email and the 9 May 2024 comparator; plus an aggregate monthly count for volume (metadata only)	Stressors 1(a) (unilateral directives) and 1(f) (disparate treatment)	Para 21 (retraction directed); Para 22 (comparable email, no discipline)	The directives' character and the disparate treatment; the count proves volume without the corpus (<i>Delaney</i>)

Item	Document sought	Matter in issue (Amended 9A)	Anchor (Form 24 admitted facts)	What the document tends to prove on s 32(5)(a)
19	Union-delegate correspondence (18 May – 18 Jun 2024; named custodians)	Stressor 1(g): suppression of industrial representation	Para 17 (QH-POL-248 "positive, supportive role"); Para 18 (notification 11 Aug 2023); Para 19 (failure to facilitate >9 months)	Whether the failure to facilitate the delegate appointment was reasonable management action

The produce-or-swear Items (1–2, 3(c), 4, 5, 7, 14, 20) bear on the same elements as sworn negatives: a hospital that cannot produce its emergency-code logs, its fatigue risk assessment, or its operational response to a complaint thereby places the very gaps the s 32(5)(a) inquiry turns on onto the record, on oath (see Part B(b)).